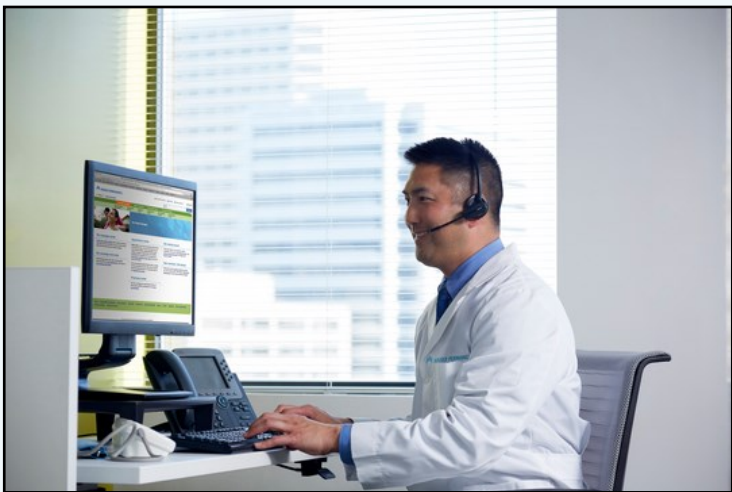




“Test Me for Everything”: HSV Testing Considerations

Clinical Case Quiz



The call center transfers a healthy 23-year-old female patient to you during a Care-on-Demand shift with a chief concern of “STD testing.” Upon further questioning, the patient indicates she is asymptomatic and reports three different male sexual partners in the last 60 days with inconsistent use of condoms. She denies receptive anal sex but endorses performing unprotected oral sex. She is taking a daily combined oral contraceptive pill and reports compliance. She states she wants to be “tested for everything.” She denies any history of painful oral or genital lesions, but states she wants to be tested for herpes, too.

On chart review, the patient’s last STD panel was 6 months ago and consisted of HIV, syphilis, hepatitis B, hepatitis C, and GC/chlamydia testing, which were negative. She has no prior history of STDs.

You should:

- a. Order HIV, syphilis, hepatitis B, hepatitis C, and GC/chlamydia testing only, and refuse to test for herpes because of the high false positive rate.
- b. Order HIV, syphilis, hepatitis B, hepatitis C, and GC/chlamydia testing as well as herpes serology, without any counseling.
- c. Order HIV, syphilis, hepatitis B, hepatitis C, and GC/chlamydia testing, as well as discuss the limitations of HSV testing. Have a shared decision-making discussion with patient on this additional testing.

The answer is c. Counseling is an important part of these clinical encounters.

Screening Considerations

Why Avoid Screening Asymptomatic Individuals?

- When used for screening asymptomatic individuals, available HSV-2 serologic tests have a low specificity and a high false-positive rate. The positive predictive value is 50%, assuming the prevalence of HSV-2 is 16%.
- Additionally, serologic testing cannot differentiate oral from genital infection.
- The US Preventive Services Task Force does not recommend testing in most asymptomatic adolescents and adults.
- There may be anxiety and disruption of personal relationships that can be associated with a positive test result.

Considerations for Select Populations

- Asymptomatic screening should be performed in pregnant persons, since those with HSV infection are at risk for shedding at the time of delivery with potential transmission to the infant.
- Do serologic testing for patients with partners who have known HSV, as this can help with the role of using antiviral therapy to reduce the risk of transmission to uninfected partners.

Testing Options

Serologic Blood Testing

- Uses a multiplex flow immunoassay.
- Run on Mondays, Wednesdays, and Fridays.
- Result turnaround time of 1 to 4 days.

Herpes “Culture” PCR

- Uses a PCR test and is the test of choice for confirming HSV infection.
- Turnaround time is 1 to 3 days.
- Requires an in-person visit with a provider to collect a swab of a suspected lesion, preferably early in the course.
- The American College of Obstetricians and Gynecologists (ACOG) has recognized that HSV PCR is 1.5 to 4 times more sensitive than viral culture.



Note. From Herpes Simplex Virus Nucleic Acid Detection, Non-CSF [Photograph], by Sanford Laboratories, n.d., Sanford Laboratories <https://sanfordlabfargo.testcatalog.org/show/NBLD0132-1>

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